

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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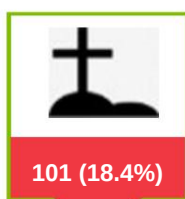
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°24/MVB_07/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	- Ituri (17/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara • - North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha - South Kivu (1/34): Miti-Murhesa
Reporting date	June 7, 2026
Publication date	June 8, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



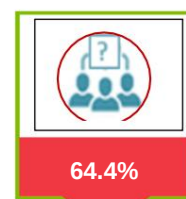
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

**Data currently being harmonized*

1. HIGHLIGHTS

Thirty -five (35) new confirmed cases, including 10 deaths, were reported on June 7, 2026, in the provinces of Ituri and North Kivu. These cases were recorded in the health zones of **Rwampara (13), Bunia (10), Mongbwalu (6) and Nyankunde (2)** in Ituri, and in the health zone **of Beni (4)** in North Kivu.

- **Seven (7) confirmed** Ebola virus disease patients at the Nyankunde Health Zone General Referral Hospital (Ituri province) have been declared cured.
- Launch of medical care activities in two (2) Ebola treatment centers (HGR) Bunia and HGR Rwampara).

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

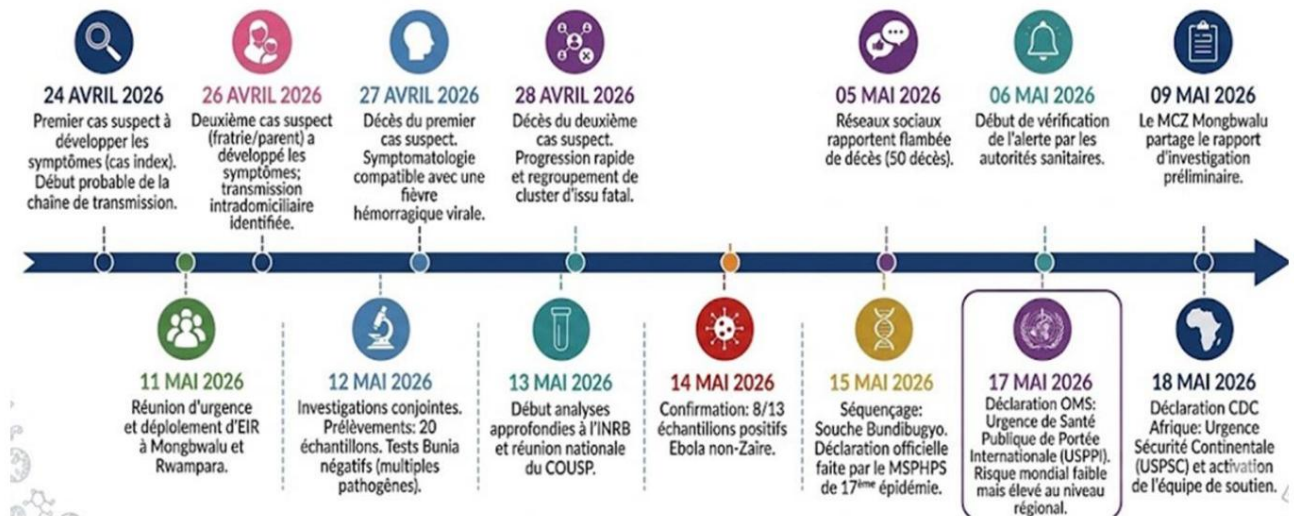


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 7, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	518	80	15.4%	17 out of 36 (47.2%)	31
North Kivu	29	20	69.0%	7 out of 34 (20.6%)	4
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	550	101	18.4%	25 out of 104 (24.0%)	35

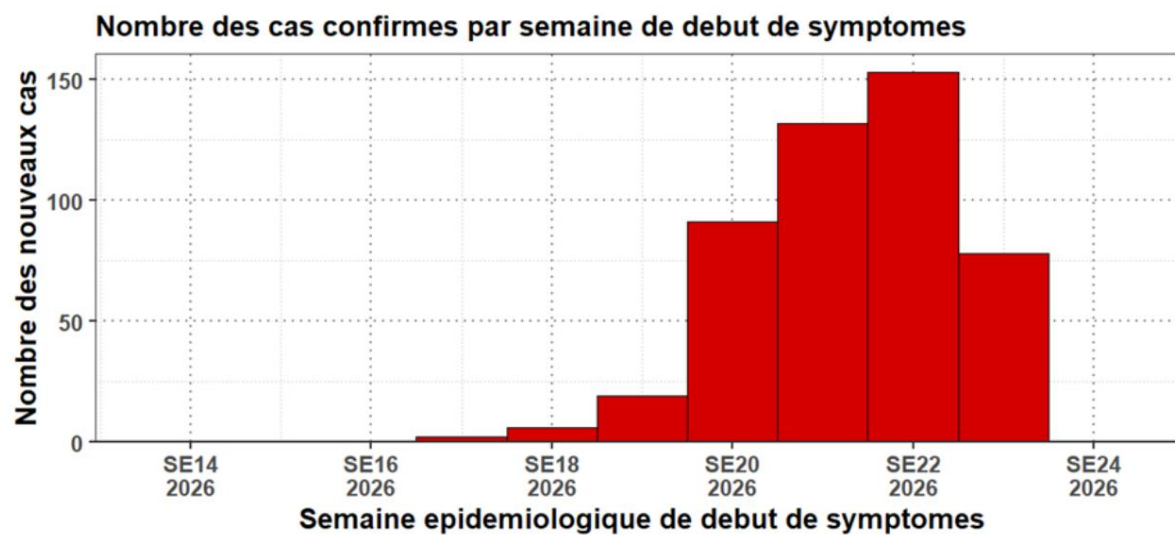
During the day of June 7, 2026, no new health zones were affected. Ituri province remains the epicenter of the epidemic, accounting for nearly 94.2% of confirmed cases among the three affected provinces. Following the recording of four new confirmed Ebola virus disease (EVD) deaths, North Kivu province now has a total of 20 deaths out of 29 confirmed cases, representing a particularly high case fatality rate of 69.0%. Conversely, despite the significant number of deaths recorded, Ituri province continues to have a relatively low case fatality rate (15.4%) considering the volume of confirmed cases.

The province of South Kivu remains the least affected, with its last confirmed case dating back to May 26, 2026.

3.2 Distribution by health zone (Cumulative as of 07/06/2026)

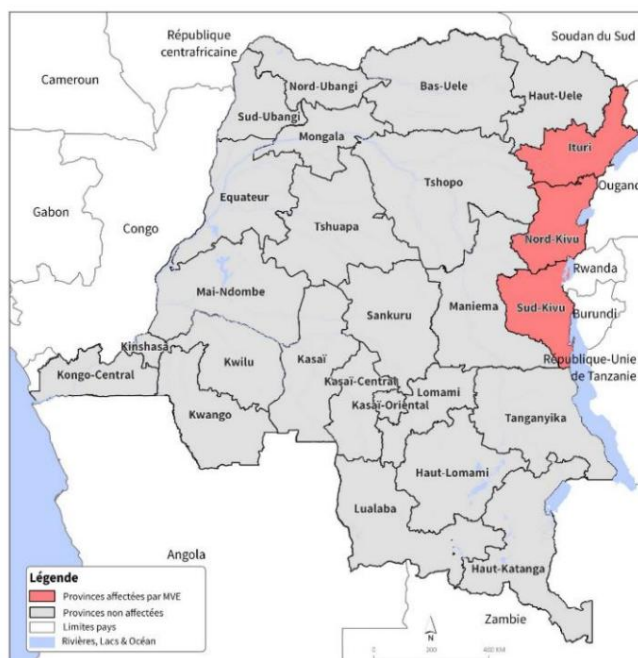
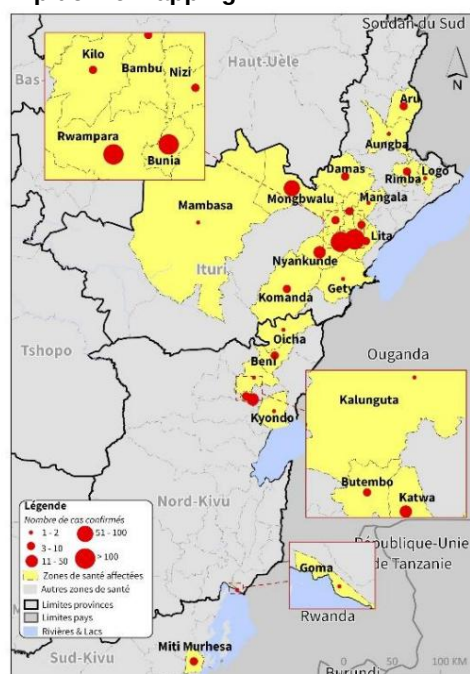
- **Ituri Province (518 cases):** Bunia (152), Rwampara (111), Mongbwalu (98), Nyankunde (26), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damas (3), Kilo (4), Nizi (5), Mangala (1), Aungba (1), Gety (1), Komanda (3), Lita (4), Logo (2). Ninety-four (94) additional cases are being ventilated.
- **North Kivu Province (29 cases):** Katwa (11), Beni (9), Butembo (4), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time-series analysis: The weekly trend remains upward despite a slight recent dip, which could be related to a delay in updating laboratory data rather than a real decrease in transmission



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 481.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 7, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 7, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI			
Bunia	152	15	9.9%
Rwampara	111	20	18.0%
Mongbwalu	98	29	29.6%
Nyankunde	26		3.8%
Bamboo	5	12	40.0%
Aru	3	1	33.3%
Kilo	4		25.0%
Nizi	5	1	0.0%
Mangala		0	0.0%
Damascus	1	0	0.0%
Aungba		0	0.0%
Gety		0	0.0%
Komanda	3113	0	0.0%
Lita	4	0	0.0%
Logo	2	0	0.0%
Mambasa	2	01	50.0%
Rimba	3	0	0.0%
Other ZS (undisclosed data)	94	10	10.6%
Ituri subtotal	518	80	15.4%
NORTH KIVU			
Katwa	11	8	72.7%
Blessed	9	7	77.8%
Butembo	4	2	50.0%
Oicha	2	2	100.0%
Kalunguta	1		100.0%
Kyondo	1	1	0.0%
Goma		0	0.0%
North Kivu subtotal	129	020	69.0%
SOUTH KIVU			
Miti-Murhesa			33.3%
South Kivu subtotal			33.3%
TOTAL	3,355	11101	18.4%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Sunday visit of the Military Governor to the Nyankunde Medical Center where a Technical Training Center is located; he encouraged Ebola patients and their family members to adhere to the acceptance of isolation at the Ebola Treatment Center when necessary.
- During the launch of the training on the zonal approach, the Director General of the INSP called on the 17 Chief Medical Officers of Health Zones of the Bunia branch as well as their Community Facilitators to actively get involved in the various activities of the response to Ebola.
- Organization of a training for all chief medical officers of zones in the Ituri province on the operation of an SGI as well as the zonal approach to be implemented for this response (60 participants: INSP/ COUSP-DRC, DPS Management Teams and ZS).
- Continued support to affected provinces under the leadership of the IM and SGI members based in Bunia, Ituri province.
- Visit to the CTE CME and Rwampara facilities by the Director General of Alima, guided by the Director General of INSP and the coordinator of COUSP.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 7, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	153	290	7	450
Alerts investigated	116	290	7	413
Investigation rate	75.8%	100.0%	100.0%	91.8%
Alerts confirmed	Alive	24	34	59
	Deceased	26	1	35
Total suspected cases for today	9 33	60	0 1	94

Based on data reported as of June 7, 2026, 450 alerts were received across the three affected provinces, of which **413 (91.8%)** were investigated. A total of **94 suspected cases were recorded** in the three affected provinces. More than half (64.4%) of the reported alerts originated from North Kivu province, 34.0% from Ituri, and 1.5% from South Kivu. Furthermore, the lowest investigation rate (75.8%) was recorded in Ituri province.

Table 3. Contact tracing of confirmed cases as of June 7, 2026

Province	Contacts under follow-up	Contacts seen (n)	Follow-up rate (%)
Ituri	(n) 4		60.1%
North Kivu	454		79.5%
South Kivu	738 226	2,678,587,224	99.1%
Total	5,418	3,489	64.4%

- Continuation of field surveillance activities (reporting alerts, investigations, active search, listing and monitoring contacts, retrospective documentary review of cases and deaths, etc.).
- Thorough investigations around confirmed cases in affected health zones.
- A total of 212 contacts have been listed in the Rwampara health zone around the confirmed cases.

- Support for the briefing of the 23 registered nurses of the Bunia Health Zone on the importance of listing and monitoring contacts through the DHIS2 tool.

Update on checkpoint and point of entry (PoC/PoE) activities

- Joint visit (Coordination of the response-PoE) to Bunia airport for a meeting with the services of the airport platform on the health protocol for travel during an epidemic.
- Conducting a simulation exercise with the various heads of land and lake border posts (Mahagi, Aru and Kasenyi), platform users and Bunia airport service providers to test the use of departure and arrival circuits after adjustments adapted to the epidemic context.
- Deployment of the sub-pillar team for the activation of the Komanda PoC on the Bunia-Kisangani axis.
- Start of construction of the huts and installation of a handwashing point at PoC Dele.

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) as of June 7, 2026

Indicators	Results
Proportion of activated PoCs	25.4% (14/55)
Number of travelers screened, % of	15,951
travelers screened, % of travelers	99.2%
refused screening, % of	0.8%
travelers who washed their hands, % of	98.6%
travelers who refused to wash their	0.4%
hands, % of travelers made aware of the risks	98.9%
Number of alerts notified	3
Number of validated alerts	3
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 63 samples collected and analyzed (positivity 49.2%; n=31)
- **North Kivu:** 183 results remain pending due to insufficient reagents.
- **South Kivu:** 4 samples received and analyzed in the laboratory, all negative for Ebola virus disease.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Evaluation scorecard in 8 health centers in the health zones of Bunia (4 health centers: scores between 11.3% and 67.7%), Aru (2 health centers: 5% and 25%), Bambu (1 health center: 58.1%) and Nyankunde (1 health center: 45.8%); beginning of the Training of the two local EDS teams (24 service providers) in the Nizi health zone. In the Bunia health zone: Briefing of 8 cleaning technicians on bio-cleaning, 20 supervisors on assessment using the scorecard tool; opening of 3 isolation units around confirmed cases in AS Aero, AS Rwambuzi and one ESS (Churches, schools, households, and social and solidarity economy (SSE) identified); decontamination of the Mudzi Hospital (internal medicine building and semi-clinical ward), 3 houses of confirmed cases, followed by the distribution of handwashing stations. In the Aru Health Zone: briefing of 5 local population representatives (LPRs) on standard precautions, the donning and doffing of PPE, and waste management; 30 ring-linked households identified. In the Rwampara Health Zone: briefing of 12 people and decontamination of 6 households of confirmed cases. In the Mongbwalu Health Zone: 6 health assessments (HAS) carried out out of 8 alerts (2 in progress due to community resistance) and 6 decontamination operations.

- **North Kivu:** Monitoring and support at CS Mitoya, CS Mukuna, CS Kyaghala in the Katwa Health Zone; closing of the training of 45 health providers from the Katwa Ebola Treatment Center; decontamination of 4/6 premises of the Horizon university clinics in the Goma Health Zone around two confirmed cases.
- **South Kivu :** Securing a body in intensive care at HGR FOMULAC Katana; support for the decontamination of the KABINDI primary school and mentoring of 4 people on the preparation of the 0.05% and 0.5% chlorine solution; briefing of 208 people from 6 health areas of the Katana and Mitimurhesa health zones on decontamination, EDS and compliance with barrier measures.

4.5 Holistic care

- Evaluation of the nutritional status of 60 children at the Saint Nicolas orphanage (Results: 2 children with SAM and 2 others with MAM).
- Launch of activities of 2 CTEs (HGR Bunia by IMC and HGR Rwampara by Alima).
- 7 new recoveries at the Nyankunde Health Zone General Referral Hospital (Ituri province). Thus, the total number of recoveries has increased from 12 to 19, including 17 in Ituri province and 2 in North Kivu (Goma and Katwa).

Table 5. Patient movement within healthcare facilities as of June 7, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	260	16	7	283
Total admissions (24h)	55	6	0	61
Exits (24h)				
Deceased (Suspects/Confessions)	9	4	0	13
No case / Cured	11	0	0	11
Escapees (Suspects/Confessions)	10	0	0	10
Transferred to the HGR	1	0	0	1
Total outflows	31	4	0	35
Patients in isolation (End of Day)	284	18	7	309
including confirmed (NC+AC)	110	5	1	116
and suspected cases	174	13	6	193

Mental and psychosocial health activities

- Psychosocial support for the two managers of the Saint Nicolas orphanage to strengthen support for 63 children, all high-risk contacts.

Table 6. Key indicators of mental health and psychosocial support activities as of June 7, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
Proportion of newly confirmed cases that benefited from SMSPS interventions	4 (12.9%)	2 (50%)	0
Proportion of confirmed cases under follow-up that benefited from SMSPS interventions	25 (28.4%)	3 (100.0%)	1 (100%)
Proportion of new suspected cases that benefited from SMSPS interventions	6 (10.9%)		5 (100%)
Proportion of suspected cases under follow-up that benefited from SMSPS interventions	24 (20.2%)	13 (86.7%)	5 (100%)
Proportion of laboratory results having been announced	48	-	0
Including positive ones	22 (45.8%)	-	0

Proportion of children separated in daycare who benefited from SMSPS interventions	11 (100.0%)	-	3 (100%)
Proportion of children separated in the community who benefited from SMSPS interventions	6 (100.0%)	-	5 (100%)
Number of accompanying persons at the CTE who benefited from interventions SMSPS	62	ND	8
Number of PPLs who benefited from SMSPS interventions	29	ND	0
Number of households and communities that benefited from SMSPS interventions	49	ND	7
Proportion of patients who benefited from the Dignity Kits at CTE			1 (100%)

4.6 Continuity of services.

- Briefing of the ESS assessment teams in the Nizi health zone.
- Evaluation of the Nizi Health Zone General Referral Hospital
- Development of recovery plans for the 3 social and solidarity economy enterprises (Komanda, Mambasa and Mongbwalu) previously evaluated.

4.7 CREC

- **North Kivu:** 255 community actors (Goma and surrounding areas) trained and briefed, including religious leaders, traditional healers, military personnel, healthcare providers, motorcyclists, municipal agents, RECO, ACSA and church leaders on risk communication, community engagement, community surveillance and rumor management; 7 rumors clarified and several community feedback documented on the existence of Ebola virus disease, the origin of the disease, the use of face masks, vaccines and Ebola virus disease treatment; 45 grassroots leaders in the Mondo health area/Katwa health zone briefed on the generalities of Ebola virus disease and the perception of Ebola virus disease in the community; 78 community leaders (45 men and 33 women) briefed on risk communication and the reporting of community alerts.
- **South Kivu:** Advocacy with 40 leaders of the health areas of Chiranga, Kalengane and Kabushwa (Katana Health Zone) and Lwiro, Kahungu and Chaoboka (Mitimurhesa Health Zone) on their involvement in the fight against Ebola virus disease; support for the briefing of 150 community relays on the Ebola virus disease package, communication techniques and CREC interventions around cases to help break the chain of transmission in the Mitimurhesa and Katana Health Zones; 7 alerts reported, including 4 alerts in the Mitimurhesa Health Zone and 3 alerts in the Katana Health Zone (1 death).
- **Ituri:** During his Sunday visit to the Nyankunde Medical Center, where an Ebola Treatment Center (ETC) is located, the military governor encouraged Ebola patients and their family members to adhere to the isolation practice at the ETC; The Director General of the National Institute of Public Health (INSP) asked the 17 chief medical officers of the Bunia health zone and their community facilitators to get involved in the various Ebola response activities (Call made during the launch of the training on the zonal approach); 10 organized mass awareness campaigns, reaching 1,819 people in 7 health zones with topics on Ebola prevention measures, the importance of the EDS, the reporting of community alerts, etc.; During home visits to 592 households, 3,738 people (1,450 men, 2,122 women, 65 boys and 101 girls) were sensitized for behavior change through 58 Community Relays (ReCos) deployed in the field.
- **Support for other pillars :** Surveillance: Contact tracing (Mongbwalu Health Center/Mongbwalu Health Zone); investigation and contact tracing (Mudzimaria Health Center/Bunia Health Zone). IPC: To prepare for community death participation in the DHS (Mudzimaria Health Center/Bunia Health Zone). Result: DHS completed.

4.8 Logistics and Security

- Monitoring of construction work at the CTE, the Bunia General Referral Hospital (finishing work in progress), and the Bunia Medical Center. HGR Rwampara (CTE).

- Delivery of PCI inputs to CS Bigo, CME Bunia (nursery), DPS and CHEM-CHEM orphanage • Delivery of medicines in the Bunia health zone.
- Monitoring of the rehabilitation work of the CTE at the Katwa General Referral Hospital.
- Continued support for the regular needs of the pillars.

4.9. Security

- **Incident: EDS team attacked** by unidentified individuals at Nyamurongo cemetery. **Summary**
2 people seriously injured and 2 vehicles damaged.

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the presence of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Awareness-raising of 822 people (**75 women, 45 men, 358 girls, 344 boys**) • Briefing of teachers in the Lita health zone on the PSEA (27 participants, including 22 men and 5 women).
- Awareness-raising on the prevention of sexual exploitation, abuse and harassment (PSEA) in the Miti Murhesa health zone under the theme: humanitarian aid is free and cannot be exchanged, sexual relations with children are prohibited, a total of 802 people including 67 women, 41 men, 357 girls and 337 boys.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3 Weakness in contact tracing (overall rate 64.4%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action	Responsible pillar	Due date
Training of service providers on the use of the RADIONE PCR	Laboratory	Day 2
1 2 Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3 Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC	Day 3

4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 60.1%)	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory		Day 2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Hospital, expansion of CREC to 15,000 households		Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for the IT/ECZS		Day 2

Training of chief physicians of health zones on the functioning of an SGI and the zonal approach, Bunia, Ituri province.



Briefing of IPC supervisors from the Bunia health zone, Ituri province.



Construction of huts begins and handwashing point installed at PoC Dele, Ituri province.



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With technical support from the World Health Organization (WHO) and
of Africa CDC